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Name: سميره محمد خليل -- National ID: 24401201900269	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



الى من يهمه الامر / مستشفى جوستاف رويس

تحية طيبة وبعد،،،

بالإشارة الى قانون الهيئة العامة للتأمين الصحي الشامل رقم 2 لسنة 2018 ، والذي يتضمن بوجوب خضوع المقيمين بمحافظات التطبيق اجباريا بالمنظومة وفقا لحل الإقامة ببطاقة الرقم القومي داخل احدي محافظات التطبيق، وبالإشارة الى بدء التشغيل الفعلي (2022\7) لمنظومة التأمين الصحي الشامل بمحافظة (اسماعيليه)

برجاء التفضل بالعلم بأنه قد تم الاستعلام عن بيانات المواطنة / سميرة محمد خليل إبراهيم تحمل بطاقة الرقم (24401201900269) المرفقه والتي قد تم اصدارها في محافظة القاهرة بناء علي ما سبق في ان المستفيد المذكور من غير الخاضعين لمنظومة التأمين الصحي الشامل

وتفضلوا بقبول فائق التقدير والاحترام،،،



تحت إشراف: 2026/02/17
رئيس الإدارة المركزية لشؤون المستفيدين



٢٠٢٥/٠٥ ٢٤٤.١٢.١٩.٠٠٢٦٩

ربة منزل



أرملة

مسلمة

أنثى

البطاقة سارية حتى ٢٠٢٢/٠٥/٠٢



سميرة

محمد خليل ابراهيم

٢٠ ش محمد عبدالحليم

حدائق القبة - القاهرة



٢٤٤.١٢.١٩.٠٠٢٦٩

LC6020306



Pathology Report

Name: سمير محمد خليل

Age: 70 years.

Clinical data: Right breast cancer (received chemotherapy.)

Specimen type: Right MRM & axillary clearance.

Date(received) : 31/12/2020

Date(reported) : 5/1/2020

Lab. Number: 2172

Hospital Number: 4794/2020 نفقة دولة

Gross Description:

MRM specimen with skin surface area; 20x13cm, including nipple and areola serial cuts revealed multiple scattered ill-defined rubbery greyish white masses measured up to 2.5x2cm, the largest was located retro areolar, located 1cm deep to skin of areola, other masses were located in the lateral half.

Least deep margins; 1.5cm.

Dissection of axillary tissue revealed 14 nodes up to 1.6x1.3cm.

Microscopic Examination :

Microscopic examination of sections prepared from the specimen received revealed two masses formed of an infiltrating malignant epithelial growth made up of groups and sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Intraductal carcinoma component 5% (high grade).

Positive for lymphovascular emboli.

There are foci of necrosis and inflammation and attempts of glandular differentiation.

Four lymph nodes out of the 14 lymph nodes detected showed metastatic deposits [Nodal score is 4/14].

The nipple, areola and surgical margins are free.

Diagnosis:

Right breast, Right MRM:

Bifocal Invasive duct carcinoma grade II. Partial pathological response to therapy.

5% high grade IDC

Free nipple , areola, deep margins.

LN status :4/14

Stage yp T2, yp N2.

Handwritten signature

Prof. Dr. Gamal Abd El Aty
Dr. Marwa Mahmoud



Patient Name: Sameera Mohamed Khalil Ibrahim	Date : 22/10/2023
Exam : C.T CHEST WITH CONTRAST	Age : 80 Year(s)
Medical ID: 4794/2020	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS: -

Compared to 7/11/2020 study report.

Chest:

- Interval left mastectomy showing no sizable operative bed masses.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Enlarged cardiac size with no pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen:

- The liver is of average size with right hepatic lobe segment VI cyst measuring 6 mm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No destructive lytic lesions.

OPINION:

- ***Tiny right hepatic cyst for follow up.***
- ***No evidence of pulmonary nodules metastatic deposits.***

Dr. Mohammed Mohsen, MSc

Dr. Ola Magdy, M.D.



Patient Name: Sameera Mohamed Khalil Ibrahim	Date : 14/08/2024
Exam : MAMMOGRAPHY BOTH	Age : 80 Year(s)
Medical ID: 4794/2020	Pat. ID: 41577

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is predominant fatty densities of the right breast (ACR a)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- Normal study of right breast (BIRADS 2)

Yours truly

Signature Dr. Asmaa Zenhom

14/08/2024 3:18 PM



Patient Name: Sameera Mohamed Khaleel

Date: 7/2/2023

Tc-99m MDP bone scan.

Desired information : skeletal surveying to detect multiple lesions.

Technique:

Delayed Anterior and posterior whole body scan 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP.

Report:

Delayed Static Phase:-

- Degenerative changes are noticed over dorso-lumbar vertebrae.
- No definite hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are normally seen.

Conclusion:

- *The surveyed skeleton is free of osseous deposits..*
- *Follow up bone scan is recommended.*

Thanks for reference

Nuclear medicine Unit

Sherien Mahdy , M.D.